- The Presenting part descends slowly and progressively.It depends on cephalopelvic relationship

c. Flexion:
   - Good flexion aids engagement and descent.

d. Internal rotation:
   - It occurs on pelvic floor. The resistance of the pelvic floor helps.
   - The head must rotate to anterior (or posterior) to pass ischial spines.

e. Crowning:
   - After internal rotation and further descent, vulva form a crown around the head (i.e.) the biparietal diameter distends the valval outlet without any recession of the head even after contraction is over.

f. Extension:
   - Distension of perineum by vertex.
   - Occiput beyond symphysis &Head stands out.

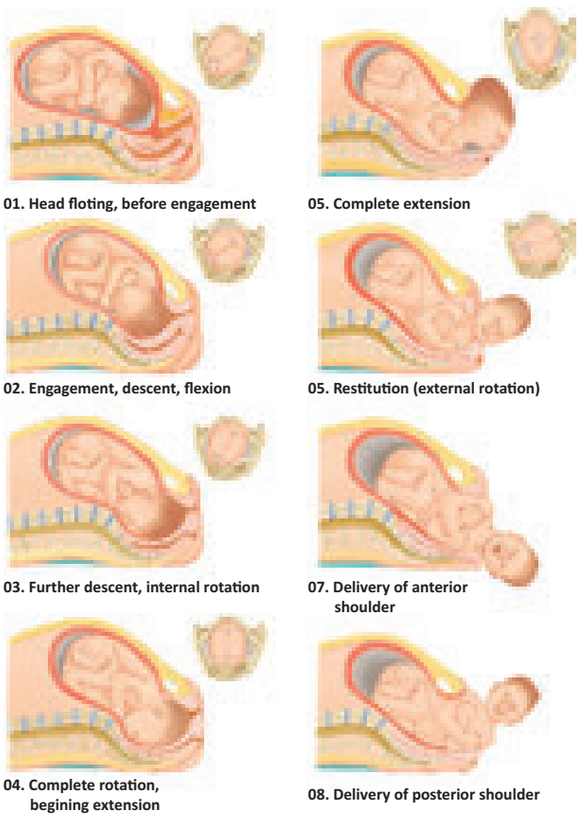
g. Restitution:
   - Untwisting of the neck, head rotates to the position occupied at engagement.

h. External rotation:
   - Shoulder descends in the path similar to that followed by the head and rotates anteroposteriorly.
   - Head swings.

i. Lateral flexion:
   - Anterior shoulder comes under the symphysis pubis.
   - Posterior shoulder sweep the perineum.

j. Expulsion:
   - After delivery of the shoulder under the symphysis pubis, the rest of the body expelled quickly. Rest of the body expelled with lateral flexion

MECHANISM OF NORMAL LABOUR



DO YOU KNOW?

At the moment a baby's born , 1/3 of their blood is still outside their body. If you delay cord clamping 90 seconds they get 60% more blood cells. They get enough iron to last them through their first year. They get white blood cells to fight infection. They get antibodies. They get stem cells to help repair their body.

7.7 Normal Puerperium

Puerperium is the period following childbirth during which the body tissues especially the pelvic organs, revert approximately to the pre-pregnant state both anatomically and physiologically.

Anatomic and physiologic changes

- Uterus involution
- Regeneration of endometrium

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